# NSS Exam Cheat Sheet

Delegation, medication safety, type 1 diabetes and anaphylaxis

## Plans and people

**Medical Alert/Support Plan** flags urgent or emergency needs. The family and school staff review it annually.

**NSS care plan** gives step-by-step directions for a student's delegated care. Delegation shares authority for a specific task.

**NSS Coordinator** develops the plan with the family and care team, delegates, trains, and monitors. The **EA** follows the plan, documents care, and is accountable to the delegating nurse.

**Principal** has final school authority. The **LST** manages the case and arranges EA assignments and training. The **school nurse** handles immunizations and supports emergency training.

**Family** provides supplies and communicates care changes. For diabetes care, text to notify and phone for directions.

Day 1 PDF pp. 8, 17–29, 88–91

## Medication safety

**Eight Rights** cover reason, medication allergies, medication, child/youth, dose, time, route, and documentation.

**Before giving.** Wash hands, check the pharmacy label and expiry, and know storage, stop date, and possible side effects.

**Medication errors** include missed doses. Notify both the parent and administrator immediately.

Day 1 PDF pp. 32–34

## Insulin and glucose monitoring

**Type 1** involves loss of insulin production. **Type 2** involves insulin resistance and may involve insufficient insulin. Insulin helps glucose enter cells.

**Conventional** doses are given at home. The **MDI** example uses a school lunch dose with a second staff check. **Pump** gives ongoing basal insulin plus meal or correction boluses.

**Pump.** Check the site before a bolus and call the parent for pump alarms. A **CGM** shows glucose trends. Follow the care plan for both.

Day 1 PDF pp. 43–49, 55–68

## Low glucose vs. high glucose

**Low** may follow extra insulin, delayed food, or more activity. Think shaky, sweaty, hungry, pale, dizzy, or unusually irritable.

**Below 4.0 mmol/L and able to swallow.** Give the fast-acting carbohydrate in the plan. Day 1 teaches a recheck after 15 minutes. Repeat treatment if still below 4.0, per plan. Stay with the student.

**Severe low** may involve a seizure, unconsciousness, or inability to swallow safely. Use the emergency plan, call 911, and give prescribed glucagon if trained. Keep the mouth clear.

**High** may involve thirst, frequent urination, flushed cheeks, drowsiness, or nausea. Follow the plan and contact the parent as directed. Allow water and bathroom access. Extra exercise can increase ketone risk.

**Ketones** can form when too little insulin leads the body to burn fat for energy. A high reading in a pump user may signal interrupted insulin delivery.

Day 1 PDF pp. 70–83. Day 2 slides 5–9

## Suspected anaphylaxis

**Recognize** breathing or swallowing trouble, hives or swelling, vomiting or cramps, or faintness. Symptoms can affect several body systems.

**Act promptly.** Give epinephrine and call 911. For the EpiPen shown, remove the blue safety release and press the orange tip to the mid-outer thigh. Follow device instructions.

**After the first dose.** Use the plan for a second dose if symptoms persist or return. Day 1 lists 5–15 minutes. Continue with ambulance transport even if the student improves. Notify the parent.

**If the student vomits,** turn the student onto their side. Epinephrine takes priority over antihistamines and asthma medication.

**Three A's.** Awareness of triggers and the plan, Avoidance of allergens, and prompt Action.

Day 1 PDF pp. 94–108

# Emergency & Daily Care

Glucagon, seizures, tube meals and asthma

## Glucagon for a severe low

**Immediate response.** Call 911, turn the student onto their side, give the prescribed glucagon if trained, and have someone call the parent. Give nothing by mouth while swallowing is unsafe.

**Powder-and-diluent kit.** Add syringe liquid to the vial, gently swirl until clear, and draw it back into the syringe. Confirm the dose in the student's emergency plan and use the prescribed product's instructions.

**Injection demo** uses the mid-outer thigh at 90 degrees. Dispose of the sharp safely without recapping. Nasal glucagon uses different product instructions.

**Afterward.** Stay with the student until EMS arrives. Once alert and able to swallow, give fast-acting carbohydrate, followed by a longer-acting snack once glucose recovers. Document and request replacement medication.

Day 2 slides 9–21

## Seizure recognition and first aid

**Seizure** is a temporary change in the brain's electrical activity. Epilepsy involves a tendency toward recurrent seizures. A focal seizure starts in one part of the brain.

**Absence** involves sudden staring. **Myoclonic** involves brief muscle jerks. **Atonic** involves loss of muscle tone. **Tonic-clonic** involves stiffening followed by jerking.

**Protect and time.** Move hazards away, protect the head, and turn the student onto their side when safe. Avoid restraint and keep the mouth clear. Stay and reassure through recovery.

**Escalate.** Follow the student's plan for emergency services and prescribed rescue medication. The course emphasizes the five-minute threshold and recurrent seizures without recovery. Breathing problems or injury also require prompt action.

**Follow-up.** Record the event in the seizure log. Contact the parent if rescue medication was given. Respiratory depression is a rare serious side effect listed in the course.

**Long-term management** may include medication, surgery, vagus nerve stimulation, or a ketogenic diet.

Day 2 slides 29–55, 59–67

## Tube meal feeding

**G-tube** delivers to the stomach. A **jejunal port** delivers to the small intestine. The **stoma** is the opening for the device.

**Continuous** is a slow, steady feed. **Bolus** is given over a shorter scheduled period. Follow the prescribed formula, rate, time, position, and flush instructions.

**Prepare and monitor.** Wash hands and use clean supplies. Check the formula's expiry. Position the student upright as directed. Flush with water as ordered to help prevent blockages.

**Suspected aspiration** may show up as coughing, choking, noisy breathing, or a wet voice. Stop the feed immediately, assess airway and breathing, and follow the emergency plan.

**Blocked tube or pump alarm.** Check for a closed clamp, kink, or twist. Follow the plan for next steps.

**Stoma changes.** Keep the site clean and dry. Report new redness, bleeding, or drainage. If the button falls out, cover the site with clean gauze, call the parent promptly, and follow the emergency plan.

**Opened formula.** Cover, mark the opening date and time, and refrigerate according to the product instructions and care plan.

Day 2 slides 76–97

## Asthma

**Typical symptoms** include cough, wheeze, chest tightness, and shortness of breath. Waking at night with symptoms suggests poor control.

**Swelling, mucus, and muscle tightening** narrow the airways. Colds, exercise, pollen, and smoke are possible triggers.

**School response.** Follow the written action plan and support quick-relief medicine as prescribed. A daily controller addresses ongoing inflammation.

**Severe distress.** If the student cannot speak or has blue lips, call 911 immediately and follow school emergency procedures.

Day 2 slides 100–109, including slide 109 speaker notes